

Mass Casualty Incident and Emergency Operations Plan Activation (Code Triage)

Policy Number	POL-EM-053	Owner / Department	Emergency Management
Effective Date	2023-07-01	Version	3.1
Last Revised	2026-03-04	Next Review	2028-03-04
Approved By	Emergency Management Committee	Applies To	All departments, the medical staff, and leadership; coordinated through the Hospital Command Center

1. Purpose

To establish a coordinated, scalable response to a mass casualty incident (MCI) or other emergency that exceeds routine capacity, through activation of the facility Emergency Operations Plan (EOP) and the Hospital Incident Command System (HICS), consistent with the CMS Emergency Preparedness Rule (42 CFR 482.15) and The Joint Commission Emergency Management standards.

2. Scope

Applies to any internal or external event producing a surge of patients or a major disruption to operations (multi-vehicle crash, industrial event, severe weather, infrastructure failure, or an on-campus mass-casualty event). The EOP is built on the six critical functions required by the Emergency Management standards: communications, resources and assets, safety and security, staff responsibilities, utilities, and patient clinical/support activities.

3. Definitions

- **Code Triage** — the overhead alert for activation of the EOP for a mass casualty or disaster; announced as *Code Triage Alert* (stand-by) or *Activation* (full response).
- **HICS** — the Hospital Incident Command System, a standardized ICS-based management structure with an Incident Commander and Operations, Planning, Logistics, and Finance/Administration sections.
- **Hospital Command Center (HCC)** — the physical or virtual location from which the incident is managed.
- **START triage** — Simple Triage And Rapid Treatment; sorts casualties into Immediate (red), Delayed (yellow), Minor/walking (green), and Expectant/deceased (black).

4. Procedure — Activation

IMPORTANT — Any charge nurse, House Supervisor, or physician recognizing a surge beyond capacity may request a Code Triage. The House Supervisor or Administrator-on-Call activates the EOP, opens the Hospital Command Center, and assumes (or assigns) the Incident Commander role until relieved. All responders report through HICS and use plain language.

1. Recognize and declare: notify the House Supervisor / Administrator-on-Call; announce Code Triage Alert or Activation overhead.
2. Open the Hospital Command Center and establish the Incident Commander and needed HICS section chiefs.
3. Decompress the ED and surge capacity: discharge/transfer eligible patients, open overflow areas, and recall staff per the labor pool.
4. Triage incoming casualties with START; direct patients to designated red/yellow/green treatment areas.
5. Manage communications, resources, and documentation through HICS; brief at regular operational-period intervals.
6. Demobilize and debrief when the surge resolves; announce all clear and begin recovery.

5. HICS Command and General Staff

HICS role	Core responsibility
Incident Commander	Overall authority and accountability; sets objectives; approves the incident action plan.
Operations Section Chief	Directs tactical response — triage, treatment, clinical care, and patient movement.
Planning Section Chief	Tracks situation status and resources; develops the incident action plan and documentation.
Logistics Section Chief	Secures staff, supplies, equipment, beds, and facility/communications support.
Finance/Administration Chief	Tracks cost, time, procurement, and claims.

6. Severity & Reporting

A mass casualty incident is a **SEV-1** event under POL-RM-013 by definition of its scope. HICS activation itself constitutes immediate executive notification; the Administrator-on-Call, Safety Officer, and Risk Management are engaged within 1 hour. Individual patient-care events occurring during the response are classified and reported on their own merits once operations stabilize. After-action review and an improvement plan are completed by Emergency Management, and the event informs the next Hazard Vulnerability Analysis.

7. Documentation Requirements

- Time of recognition, declaration, and command-center activation; Incident Commander of record.
- HICS organization chart and section assignments (HICS 203/207).
- Operational-period objectives and incident action plans (HICS 201/202).
- Casualty count by triage category and disposition; bed and resource status.
- Demobilization, all-clear time, and the after-action report / improvement plan.

8. References

- CMS Emergency Preparedness Rule, 42 CFR 482.15 (EOP, training/testing, communications).
- The Joint Commission, Emergency Management (EM) chapter — six critical functions.
- HHS ASPR, Hospital Incident Command System (HICS) Guidebook and forms.

- Related: POL-RM-013 (severity & reporting), POL-EM-050 through POL-EM-057 (code-specific response).